



DIPHENMIN LINCTUS

Description : Dark brown, raspberry flavoured viscous solution

Content

Diphenhydramine hydrochloride 14 mg / 5 ml

Preservative : Sodium Benzoate 0.1% w/v

Alcohol 0.019% v/v

Indications

Control of cough and alleviation of nasal stuffiness, sneezing, lachrymation and bronchial congestion.

Pharmacology:

Diphenhydramine hydrochloride is an ethanolamine derivative with the properties and uses of the antihistamines. It is less potent than promethazine hydrochloride and has a shorter duration of action, but like promethazine, it has pronounced sedative properties. It also has antiemetic, anticholinergic and local anaesthetic properties. It diminishes and abolishes the main actions of histamine in the body, probably by supplying the H1 receptor sites in the effector cells to the exclusion of histamine. It does not prevent the production of histamine.

Diphenhydramine is readily absorbed after oral administration. The plasma half life of an oral dose is 13 - 21 hours. It is extensively metabolised in the liver before reaching the general circulation. Reactions include N-dealkylation and deamination. The major urinary metabolite appears to be diphenylmethoxy acetic acid in free or conjugated forms.

Adverse Effects:

The most common side effect is sedation, dizziness, lassitude, hypotension, muscular weakness. Other side effects include gastrointestinal disturbances such as nausea, vomiting and diarrhoea or constipation. It can also cause headache, blurred vision, tinnitus, elation or dryness of mouth.

Precautions

It may cause drowsiness and dulling of mental alertness. Patient should not take charge of vehicles or machinery where loss of attention may lead to accidents. Patients should abstain from alcohol.

General: Diphenhydramine hydrochloride has an atropine-like action and therefore should be used with caution in patients with a history of bronchial asthma, increased intraocular pressure, hyperthyroidism, cardiovascular disease or hypertension.

Warning:

Antihistamines should be used with considerable caution in patients with narrow angle glaucoma, stenosing peptic ulcer, pyloroduodenal obstruction, symptomatic prostatic hypertrophy or bladder neck obstruction.

Use in children: In infants and children, especially, antihistamines in overdose may cause hallucinations, convulsions, or death. As in adult, antihistamines may diminish mental alertness in children. In the young child, particularly, they may produce excitation.

Use in the elderly (approximately 60 years or older)

Antihistamines are more likely to cause dizziness, sedation and hypotension in elderly patients.

- Not to be used in children less than 2 years of age.

- To be used with caution and on doctor's or pharmacist's advice in children 2 to 6 years of age.

Contraindications:

- Diphenhydramine should not be used in premature or full term neonates, because it can increase antimuscarinic effects in these group like CNS excitation.

- Use in nursing mothers: Because of the higher risk of antihistamines for infants generally, and for newborns and premature in particular antihistamine therapy is contraindicated in the following conditions. Hypersensitivity to diphenhydramine hydrochloride and other antihistamine of similar chemical structure.

Drug Interactions:

a) Alcohol or CNS depression producing medications, other

(concurrent use may potentiate the CNS depressant effects of either these medications or antihistamines; also concurrent use of maprotiline or tricyclic antidepressants may potentiate the anticholinergic effects of either antihistamines or these medications)



- b) Anticholinergic or other medications with anticholinergic activity
(anticholinergic effects may be potentiated when these medications are used concurrently with antihistamines, patients should be advised to report occurrence of gastrointestinal problems promptly since paralytic ileus may occur with concurrent therapy)
- c) Apomorphine
(prior administration of diphenhydramine may decrease the emetic response to apomorphine in the treatment of poisoning)
- d) Monoamine oxidase (MAO) inhibitors, including furazolidone, pargyline and procarbazine.
(concurrent use of MAO inhibitors with antihistamines may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines, concurrent use is not recommended.)
- e) Ototoxic Medications
(concurrent use with antihistamines may mask the symptoms of ototoxicity such as tinnitus, dizziness or vertigo)
- f) Photosensitising medications, other
(concurrent use of these medications with antihistamines may cause additive photosensitizing effects)

Overdosage

Since there is no specific antidote for overdose with antihistamines, treatment is symptomatic and supportive with possible utilization of the following:

- * Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration necessary, especially in infants and children.
- * Gastric lavage (isotonic or 0.35% sodium chloride solution) if patient unable to vomit within 3 hours of ingestion.
- * Saline cathartics (milk of magnesia) are sometimes used.
- * Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure.
- * Oxygen and intravenous fluid.
- * Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Dosage and Administration

For oral use.

Adults: 1 - 2 teaspoon 3 - 4 times daily.

Children: 1/2 - 1 teaspoon 3 - 4 times daily.

Storage : Keep container tightly closed.
: Store in a cool, dry place below 30°C.
: Protect from light. Keep out of reach of children

Pack size : 90ml/bottle

Shelflife : 3 years

Product registration : PRIME PHARMACEUTICAL SDN. BHD.

holder & Manufacturer : 1505, Lorong Perusahaan Utama 1, Taman Perindustrian Bukit Tengah,
14000 Bukit Mertajam, Penang, Malaysia.